

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: Real-World Treatment Patterns and Outcomes of Advanced Renal Cell Carcinoma (aRCC) Patients Receiving Nivolumab+Ipilimumab or Pembrolizumab+Lenvatinib in U.S. Oncology Practice.

Short Title/Acronym: CA209-1275 aRCC Real-World Outcomes Study

Protocol Number: CA209-1275

NCT Number: NCT06345183

Sponsor Name: Bristol-Myers Squibb

Investigational Product: Combination Product : Nivolumab + Ipilimumab combination therapy; Combination Product : Pembrolizumab + Lenvatinib combination therapy

Phase of Development: Observational (N/A for Phase)

Medical Monitor: Bristol-Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Brief Summary This observational study aims to describe demographic, clinical characteristics, treatment patterns outcomes of participants with advanced Renal Cell Carcinoma (aRCC) receiving either Nivolumab + Ipilimumab, or Pembrolizumab + Lenvatinib combination therapy.

2.2 Study Objective Primary Objective: To evaluate the real-world outcomes of first-line Nivolumab + Ipilimumab combination therapy compared to Pembrolizumab + Lenvatinib combination therapy among patients with Advanced Renal Cell Carcinoma (aRCC) in the United States. **Secondary Objectives:** To assess real-world overall survival (rwOS), real-world progression-free survival (rwPFS), time to next treatment, and real-world safety profiles of the respective combination therapies.

2.3 Background & Rationale To address the gap in comparative effectiveness data in real-world clinical practice for advanced Renal Cell Carcinoma (aRCC). While both Nivolumab+Ipilimumab and Pembrolizumab+Lenvatinib are established first-line treatments for aRCC, generating real-world evidence (RWE) is medically necessary to understand treatment patterns, effectiveness, and survival outcomes outside of controlled clinical trial settings among US patients.

3. Study Design & Methodology

3.1 Design Framework Study Type: Observational (Retrospective Cohort Study) **Control Type:** Active Comparator (Real-world

cohorts) **Blinding:** Open-label (N/A for retrospective observational) **Randomization:** N/A (Observational)

3.2 Planned Enrollment & Duration Target **\$N\$:** Not specified (All eligible patients identified in the US database meeting criteria) **Number of Sites:** Multiple (US-based Electronic Medical Records/ Database) **Study Start (Actual):** 19-Feb-2024 **Primary Completion (Actual):** 04-Apr-2024 **Study Completion (Actual):** 04-Apr-2024

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years and older with a diagnosis of Advanced Renal Cell Carcinoma (aRCC) with predominantly clear-cell histology. 2. Received any of the following therapies in the first-line setting: Nivolumab + Ipilimumab between 8/1/2021 and 6 months prior to data collection, OR Pembrolizumab + Lenvatinib between 8/1/2021 and 6 months prior to data collection. 3. Participant had at least 6 months of follow-up from initiation of index treatment (patients who deceased prior to 6 months since index treatment initiation would still be eligible).

4.2 Exclusion Criteria 1. Any active malignancy in the 3 years prior to initiation of 1L therapy for aRCC, except for locally curable cancers that have been cured, such as basal or squamous cell skin cancer, superficial bladder cancer or carcinoma in situ of the cervix, or breast cancer, or localized prostate cancer with a Gleason score $\leq 3+4$ that has been treated more than 12 months prior to full study screening and considered to have a very low risk of recurrence. 2. Receipt of any immunotherapy or tyrosine kinase inhibitor (TKI) therapy as part of a Randomized Controlled Trial (RCT). 3. Participant received any systemic therapy for Advanced Renal Cell Carcinoma (aRCC) prior to index therapy, including neoadjuvant or adjuvant therapy.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Standard real-world dosing as prescribed by the treating physician for Nivolumab + Ipilimumab OR Pembrolizumab + Lenvatinib. **Route of Administration:** Intravenous (IV) for Nivolumab, Ipilimumab, and Pembrolizumab; Oral for Lenvatinib. **Duration of Treatment:** Retrospectively assessed duration from therapy initiation until disease progression, unacceptable toxicity, or end of real-world data collection period.

5.2 Concomitant & Prohibited Therapy Permitted: Standard of care for aRCC and supportive therapies as documented in real-world records. **Prohibited:** Prior systemic therapy for aRCC (including neoadjuvant or adjuvant) and participation in an RCT for immunotherapy or TKI therapy.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	N/A	Database screening for eligible aRCC index treatments (from 01-Aug-2021)
Baseline	Day 0	Initiation of first-line index therapy (Nivolumab+Ipilimumab or Pembrolizumab+Lenvatinib)
Interim	N/A	Retrospective data extraction of routine clinical visits and assessments
End of Study	Follow-up	Minimum 6 months post-initiation of index treatment (or until death)

(Note: As a retrospective observational study, no prospective patient visits are scheduled.)

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Real-world clinical outcomes, notably comparative real-world Overall Survival (rwOS) and real-world Progression-Free Survival (rwPFS) between the two treatment cohorts. **Measurement Method:** Analysis of abstracted Electronic Health Records (EHR) and oncology databases.

7.2 Secondary & Exploratory Endpoints Real-world Overall Response Rate (rwORR) Time to Next Treatment (TTNT) Safety and incidence of real-world treatment-emergent adverse events (TEAEs)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Retrospective abstraction of documented adverse events from electronic medical records and clinical notes. **Serious Adverse Events (SAEs):** Abstracted historically as recorded during hospitalizations or emergency visits linked to the index therapies. **Data Monitoring Committee (DMC):** Internal review by the Sponsor's Health Economics and Outcomes Research (HEOR) team.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: All eligible aRCC patients meeting the specific inclusion/exclusion criteria identified in the database (Real-World Analysis Set). **Sample Size Justification:** Dependent on the maximum number of eligible patients available in the database between August 1, 2021, and the data cutoff. **Handling of Missing Data:** Appropriate statistical imputation techniques for handling missing real-world data or variables missing completely at random (MCAR), as defined in the study protocol.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study was conducted in accordance with Good Pharmacoepidemiology Practices (GPP) and local patient data privacy regulations (e.g., HIPAA compliance for de-identified data). **Monitoring Plan:** Quality control mechanisms applied to data abstraction, including validation rules for the EHR database. **Audit Trail:** Secure database access logs and strict protocol adherence for retrospective data lock.

11. Study Identifiers & Governance

Primary ID: CA209-1275 **Registry Number:** NCT06345183 **Sponsor/Lead Organization:** Bristol-Myers Squibb **Study Title:** CA209-1275 aRCC Real-World Outcomes Study **Official Regulatory Title:** Real-World Treatment Patterns and Outcomes of Advanced Renal Cell Carcinoma (aRCC) Patients Receiving Nivolumab+Ipilimumab or Pembrolizumab+Lenvatinib in U.S. Oncology Practice. **Last Update Posted:** 06-Nov-2025

12. Clinical Framework (aRCC Specific)

Primary Condition: Advanced Renal Cell Carcinoma (aRCC) **Diagnosis Threshold:** Histologically confirmed predominantly clear-cell aRCC **Medical Methodology:** Retrospective First-Line Immunotherapy/TKI Evaluation **Optimization Target:** Real-world survival and effectiveness metrics

13. Study Procedures & Methodology

Management Pathway: Retrospective EHR data abstraction and analysis **Education Protocol (HCP):** N/A (Retrospective observational study) **Education Protocol (Patient):** N/A (Retrospective observational study) **Quality Audit Mechanism:** Algorithmic and manual data curation tracking from validated real-world datasets

14. Observation & Follow-Up Schedule

Total Duration: Minimum 6 months of follow-up from index date **Onsite Visit Cadence:** N/A (Historical chart review) **Remote Monitoring Frequency:** Continuous EHR data extraction matching clinical history **Checklist Review Interval:** Single comprehensive data pull per patient

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]					